

ENCOUNTER PROGRESS NOTE

PATIENT

Name: Mildred Reyes
DOB: 1947-06-07
MRN: MRN-2026-0407
Date of Service: 2026-04-15
Provider: Midwest Cardiac & Specialty Group
Visit Type: Office / Outpatient

CHIEF COMPLAINT

Acute shortness of breath and lower-extremity swelling worsening over 4 days, with orthopnea and paroxysmal nocturnal dyspnea.

HISTORY OF PRESENT ILLNESS

Mrs. Reyes is a 78-year-old female with longstanding heart failure who presents with progressively worsening dyspnea on exertion, now occurring with minimal activity (NYHA Class III). She reports 3-pillow orthopnea and PND that wakes her twice nightly. She has gained 6 lbs in 5 days and notes increased bilateral pedal edema up to the knees. No chest pain. Denies fever, cough, or recent travel.

PAST MEDICAL HISTORY

Heart failure with reduced ejection fraction (HFrEF), hypertension, type 2 diabetes mellitus controlled on metformin. Prior MI 2019.

PHYSICAL EXAMINATION

BP 158/92, HR 98, RR 22, SpO2 92% on RA. Jugular venous distention to angle of jaw. Bilateral basilar crackles. S3 gallop audible. Bilateral 2+ pitting edema to the knees. No acute distress at rest.

LABS / IMAGING

NT-proBNP elevated at 6,200 pg/mL (baseline ~1,800). Troponin negative x2. Creatinine 1.3, BUN 28. Echocardiogram performed: LVEF 30% (severely reduced), moderately dilated left ventricle, no regional wall motion abnormality beyond globally reduced function. Mild MR. ECG: sinus tachycardia, LBBB unchanged from prior. Chest X-ray: cardiomegaly with pulmonary vascular congestion and small bilateral pleural effusions.

ASSESSMENT

- Acute decompensation of chronic systolic heart failure. The documented LVEF of 30% with new NYHA Class III symptoms, elevated NT-proBNP, and radiographic congestion support an acute systolic (congestive) heart failure exacerbation (I50.21).
- Hypertension, suboptimally controlled today in setting of volume overload (I10).

PLAN

IV furosemide 80 mg IV now, then 40 mg IV BID. Strict I/O monitoring. Daily weights. Increase carvedilol once euvolemic. Cardiology consult completed; agrees with management. Consider further uptitration of guideline-directed medical therapy on discharge.

PROVIDER SIGNATURE

Electronically signed by Dr. Elena Vasquez, MD on 2026-04-15.